**This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.**

Patient Group Direction
for the supply of Dapoxetine 30mg/60mg tablets (Priligy) for the treatment of Premature Ejaculation

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

## Summary of the governing guidance for premature ejaculation: the Priligy (dapoxetine) SmPC, the BNF, and the European Association of Urology guidelines on sexual and reproductive health (2024). There is no NICE or NICE CKS topic on premature ejaculation.

### Overview

Premature ejaculation (PE) is defined as persistent or recurrent ejaculation with minimal stimulation before the individual desires it. PE can be lifelong (present since the first sexual experiences) or acquired (developing after a period of normal sexual function).

### Assessment

- **Sexual history:** Obtain a detailed sexual history including intravaginal ejaculatory latency time (IELT), impact on patient and partner, and relationship factors.
- **Psychological factors:** Screen for anxiety, depression, and relationship difficulties that may contribute to or result from PE.
- **Medications:** Review current medications that may affect sexual function (e.g., SSRIs, other serotonergic agents).

- **Medical conditions:** Exclude underlying prostatitis, thyroid dysfunction, and other neurological or urological conditions.

**Management**

- **First-line approach:** Behavioural techniques including stop-start and squeeze techniques, cognitive behavioural therapy (CBT), and couples counselling are recommended before pharmacological intervention.
- **Pharmacological options:** Dapoxetine is an on-demand SSRI approved for the treatment of premature ejaculation in men aged 18-64 years.

**Red Flags**

- Underlying prostatitis - refer to GP/urology
- Thyroid dysfunction - check thyroid function tests
- Neurological conditions - refer to specialist if suspected
- Severe relationship distress - consider psychological/couples therapy referral
- Under this PGD, symptoms suggesting prostatitis, thyroid dysfunction or a neurological cause, and premature ejaculation of recent onset with another new symptom, are exclusion criteria: refer for a diagnosis, do not supply.

# Patient Group Direction

for the supply of

**Dapoxetine 30mg/60mg tablets (Priligy)**

for the treatment of

**Premature Ejaculation**

**Healthcare professionals covered and training**

| | **Healthcare professionals allowed to work under this PGD and requirements** |
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| **Qualifications and Registration** | • Pharmacist registered and practicing with the GPhC<br>• Pharmacy technician registered and practicing with the GPhC |
| **Training and assessment** | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>• All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>• All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>• All users should complete the Get Real Health training for this |

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| | PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>• Must work in compliance with the SOPs of their own employer<br>• All users must have appropriate indemnity in place to cover this service |
| **Further training and responsibilities** | • All users must be up to date with CPD requirements and appraisal<br>• All users must be up to date with MHRA and safety alerts with regards to medical products<br>• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005. |

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| **PGD Indication** | Treatment of premature ejaculation in adult men aged 18-64 years presenting with a history of persistent or recurrent ejaculation with minimal stimulation that causes personal distress. |
| **Inclusion criteria** | Adult males aged 18-64 years inclusive<br>Intravaginal ejaculatory latency time (IELT) of less than 2 minutes<br>Premature ejaculation causing significant personal distress<br>Patient has provided informed written consent<br>Able to understand and comply with instructions |
| **Exclusion criteria** | Age <18 years or ≥65 years<br>Significant cardiac disorders including NYHA class II-IV heart failure<br>Conduction abnormalities or conditions that prolong QT interval<br>History of ischaemic heart disease<br>Significant valvular disease<br>History of syncope or orthostatic hypotension<br>Concurrent use of MAOIs, thioridazine, SSRIs, SNRIs, tricyclic antidepressants, or any other serotonergic medicine including tramadol, triptans, linezolid, lithium, L-tryptophan and St John's wort, or use of any of these within the last 14 days |

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| | Moderate or severe hepatic impairment (Child-Pugh class B or C)<br><br>Moderate or severe renal impairment (creatinine clearance below 50 mL/min; severe is below 30 mL/min)<br><br>Concurrent use of potent CYP3A4 inhibitors (ketoconazole, ritonavir, etc.)<br><br>History of bipolar disorder or mania<br><br>Symptoms suggesting prostatitis (perineal, pelvic or genital pain, painful ejaculation, dysuria or lower urinary tract symptoms), thyroid dysfunction (weight change, heat or cold intolerance, palpitations, tremor, marked fatigue) or a neurological cause (new numbness, weakness, or bladder or bowel symptoms): refer for a diagnosis before any SSRI is supplied<br><br>Premature ejaculation of recent onset together with another new symptom: refer for a diagnosis |
| **Cautions** | Mild hepatic impairment (Child-Pugh class A) - use with caution; moderate or severe impairment excludes<br><br>Moderate CYP3A4 inhibitors (for example erythromycin, clarithromycin, fluconazole, diltiazem, verapamil) - increase dapoxetine concentrations; 30mg only, do not increase to 60mg<br><br>CYP2D6 poor metabolisers, known or suspected - 30mg only, do not increase to 60mg (SmPC)<br><br>Risk factors for orthostatic hypotension - advise to stand slowly<br><br>Concomitant use of PDE5 inhibitors (sildenafil, tadalafil) - increased hypotension risk<br><br>Bleeding disorders or concurrent anticoagulant therapy - increased bleeding risk<br><br>History of seizures - dapoxetine may lower seizure threshold<br><br>Hyponatraemia - monitor sodium levels, particularly during first 2 weeks |
| **Actions if patient is excluded or declines treatment** | Advise on alternative treatment options and how these can be accessed.<br><br>Document any advice given and the decision reached. Inform or refer to the GP as appropriate. |

***Details of the medicine***

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| **Name, form and strength of medicine** | Dapoxetine 30mg and 60mg tablets |
| **Legal category** | POM |
| **Storage** | Store below 30°C. Keep in original container. Keep out of reach of |

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| | children. |
| **Route/method of administration** | Oral - swallow tablets whole with water. May be taken with or without food. |
| **Dose and frequency** | Starting dose: 30mg orally, taken 1-3 hours before anticipated sexual activity<br>May be increased to 60mg if the 30mg dose is insufficient and well tolerated<br>Maximum: one dose per 24-hour period<br>Do not take on a daily basis; use only as needed before sexual activity |
| **Quantity to be administered and/or supplied** | Up to 6 tablets per supply (sufficient for up to 6 doses) |
| **Maximum or minimum treatment period** | Review efficacy and tolerability after 4 weeks of use (approximately 6 doses). Reassess every 6 months if continuing treatment. If inadequate response after 6 doses at the recommended dose, consider referral to GP or specialist. |
| **Adverse effects** | Very common (>1 in 10): Headache, dizziness, nausea<br>Common (1 in 10 to 1 in 100): Somnolence, insomnia, fatigue, anxiety, diarrhoea, vomiting, dry mouth, erectile dysfunction, nasopharyngeal congestion, sinus congestion<br>Uncommon (1 in 100 to 1 in 1,000): Hypotension, orthostatic hypotension, syncope, palpitations, tremor, paraesthesia<br>Rare (<1 in 1,000): Seizures, serotonin syndrome (with concurrent serotonergic drugs), hyponatraemia, angle-closure glaucoma |

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| **Yellow card reporting** | Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. |
| **Records to be kept** | Record the following information:<br>• that valid informed written consent was given<br>• name of individual, address, date of birth and GP<br>• name of healthcare practitioner<br>• name and brand of medication<br>• date of supply dose, form and route quantity supplied<br>• that the exclusion questions on symptoms suggesting prostatitis, thyroid dysfunction or a neurological cause, and on recent onset with another new symptom, were asked and the answers given<br>• advice given, including advice given if excluded or declines treatment |

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| | • details of any adverse drug reactions and actions taken supplied via PGD<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.<br>A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:<br>• Adults aged 18 years and over - keep records for 8 years<br>• Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

***Patient information***

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| **Written information to be given to patient or carer** | Supply the patient information leaflet (PIL) provided with Priligy or dapoxetine product. Ensure patient understands the intended use as an on-demand medication before sexual activity. |
| **Follow-up advice to be given to patient or carer** | Take the tablet 1-3 hours before sexual activity as directed - do not take on a daily basis<br>Avoid concurrent use of alcohol as this increases the risk of dizziness and syncope<br>Stand up slowly from sitting or lying down, particularly after the first dose, to minimise dizziness<br>Maintain adequate hydration, especially in warm weather<br>Do not drive or operate machinery if experiencing dizziness or somnolence<br>Report any symptoms of chest pain, severe headache, or fainting immediately to your GP<br>If you experience an erection lasting longer than 4 hours (priapism), seek immediate medical attention<br>Inform your GP if you are using this medication, particularly before starting any new medications<br>Do not exceed one dose per 24-hour period<br>If symptoms do not improve after 6 doses, discuss with your healthcare provider or GP for further assessment |

## Key references


- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources

- European Association of Urology, Guidelines on Sexual and Reproductive Health, 2024 edition, section on premature ejaculation; Priligy Summary of Product Characteristics, current version; BNF, dapoxetine
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

**Agreement to practise by the Registered Healthcare Professional**

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

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| Name and address of pharmacy premises / clinic premises to which this PGD relates | | | |
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| Name of healthcare professional | Job title | Registration number | Signature |
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| Valid from date | Expiry date | | |
| 30/9/26 | 31/7/27 | | |

**PGD adoption and authorisation by the employer/clinical lead**

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
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| Name of professional group signatory | Job title & name of organisation | Signature | Date |
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**Change history to version 001**

Later versions are recorded in the Version and change record at the end of this document.

| **Version number** | **Change details** | **Date** |
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| 001 | Development & issue of new PGD | 1st November 2025 |
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**Version and change record**

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

**Previous version records**

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 46: the guidance summary red flags are now exclusion criteria: symptoms suggesting prostatitis, thyroid dysfunction or a neurological cause, or premature ejaculation of recent onset together with another new symptom, refer for a diagnosis before any SSRI is supplied; the guidance summary says so and the records row asks for the answers given.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document

above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover and arm heading corrected from administration to supply.

Renal exclusion threshold matches its label: moderate or severe impairment is creatinine clearance below 50 mL/min; the previous figure (below 30) was severe only.

Moderate CYP3A4 inhibitors and CYP2D6 poor metabolisers: the dose adjustment is stated, 30mg only, as the SmPC directs.

Records require written consent, matching the inclusion criterion.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references are attributed to the SmPC, the BNF and the EAU guideline. There is no NICE CKS topic on premature ejaculation; the previous version cited one.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

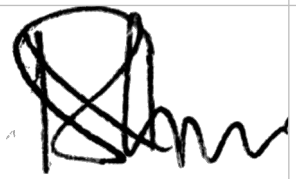
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Hepatic impairment: the Priligy SmPC contraindicates moderate as well as severe impairment (Child-Pugh B and C). The previous version excluded class C only and permitted moderate impairment under a caution. Now excluded; caution applies to mild (class A) only.

Serotonergic medicines: the exclusion named MAOIs, thioridazine, SSRIs, SNRIs and tricyclics. The SmPC contraindication covers all serotonergic medicines and applies for 14 days after stopping them. Tramadol, triptans, linezolid, lithium, L-tryptophan and St John's wort added, with the 14 day washout.

## Signed on behalf of Get Real Health

Version v007, 30 September 2026.

| Name | Qualification | Signature | Date |
|---|---|---|---|
| Nitin Shori | Medical Director<br>GMC: 6047293 |  | 30 September 2026 |
| Chris Pilkington | Head Pharmacist<br>GPhC: 2046322 |  | 30 September 2026 |

*Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.*